

Supplementary Tables 1, 2 and 3

Supplementary Table 1: Day-to-day context for engaging in podiatry for both Pre-FC and Post-FC participants

SUPPLEMENTARY TABLE 1. Median number of acquired points for the initial rotor map with each mapping system.

<u>Motivation</u>	<u>Group</u>	<u>Median (IQR)</u>	<u>Mean rank Precision*</u>	<u>U</u>	<u>z</u>	
<u>Contribution to society</u>	<u>Pre-FC</u>	5 (4-5)	55.09	1479.5	-1.059	0
	<u>Post-FC</u>		60.24			
	<u>Pre-FC</u>	22135	5 (5-5)	2576		
<u>Skills to improve quality</u>	<u>Left atrium</u>			1476.5	-1.226	
		{12603– 30902}	{2368– 6019}	55.035452– 6426		
(number of lifemaps = 17)	<u>Post-FC</u>		60.28			
<u>Build human relationships</u>	<u>Pre-FC</u>	5 (4-5)	58.03	1623.5	-0.010	0
	<u>Post-FC</u>		57.98			
<u>Work with a range of service</u>		5 (4-5)	2372			
<u>users</u>	<u>Pre-FC</u>	11546				
	<u>Right atrium</u>		–2427	1613	-0.082	
		{10380– 16678}	57.762317– 3292			
(number of maps = 9)	<u>Post-FC</u>		58.18			
<u>Work with specific groups</u>	<u>Pre-FC</u>	4 (4-5)	58.85	1582.5	-0.256	0
	<u>Post-FC</u>		57.35			
<u>Fulfilling</u>	<u>Pre-FC</u>	5 (4-5)	54.22	1436	-1.393	0
	<u>Post-FC</u>		60.91			
<u>Working under pressure</u>	<u>Pre-FC</u>	4 (4-5)	59.78	1536	-0.544	0
	<u>Post-FC</u>		56.63			
<u>Non-sedentary profession</u>	<u>Pre-FC</u>	4 (3-5)	56.98	1574	-0.303	0
	<u>Post-FC</u>		58.78			
<u>Work in a team</u>	<u>Pre-FC</u>	4 (4-5)	58.31	1609.5	-0.096	0
	<u>Post-FC</u>		57.76			
<u>Offers variety</u>	<u>Pre-FC</u>	5 (4-5)	56.36	1543	-0.542	0
	<u>Post-FC</u>		59.26			

Note: IQR, * Points in use. Total number of points was 37183 – 92480 for the left atrium, and 14296 – 19785 for the right atrium.

Data are shown as median [interquartile range (25th – 75th percentiles); Pre-FC, before first career; Post-FC, after first career] or as range, if only two maps were created.

Supplementary Table 2: How information on podiatry is accessed for both Pre-FC and Post-FC participants

SUPPLEMENTARY TABLE 2. Detailed procedural results in each patient.

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Health Website

Pre-
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Post-FC

55.13

Job Website2

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0.37 2 Small reentry in the RSPV+ right superior PVL.

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Post
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4	3	±	AT			-Typical atrial flutter. -Focal AT in the posterior LA wall.
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5	3	AT			-Perimitral flutter: ASL. -Typical atrial flutter. -SVC isolation + line between SVC and IVC to connect rotor ablation sites.
6	2	AT			-Small reentry in the LA roof. -Perimitral flutter: MAL. -Typical atrial flutter.
7	2	SR			-Macroreentry around the left pulmonary veins. -Macroreentry around the right pulmonary veins.

8	5	SR			-PVI (rotors within the RSPV and LSPV antra). -Focal AT in the crista terminalis.
9	3	AT			-Small reentry in posteroseptal mitral annulus. -Perimitral flutter: MAL. -Focal AT from proximal coronary sinus. -PVI (gaps from previous procedure). -Prophylactic ASD. -CTI ablation (gap from previous procedure).

		<u>1</u> (<u>1-1</u>)	<u>15</u> <u>20</u>	<u>-</u> <u>0.6</u> <u>29</u>	<u>0.5</u> <u>30</u>	<u>0.0</u> <u>62</u>	AT	-Typical atrial flutter.
								-Macroreentry around the left pulmonary veins.
								-Right inferior PVI (gap from previous procedure).
<u>University Website</u>	<u>Pre-FC</u>	<u>59</u> <u>.1</u> <u>0</u>		<u>Post-FC</u> <u>0</u> (<u>0-0</u>)		<u>56.25</u>		<u>0.754</u> -Right PVI (sites with SPT within the RSPV antrum).
								Left pulmonary veins were spontaneously isolated.
								-Line from anterior mitral annulus to
								scar in the anterior LA
<u>Social media</u>	<u>Pre-FC</u>				<u>58.32</u>	<u>15594</u>	<u>-</u> <u>0.314</u> <u>No</u> <u>effect</u>	

						wall including site with SPT. —Empirical CTI ablation after electrical cardioversion. - Right PVI (rotor withi n the RSPV antru m). - Small reent ry in the poste rior LA roof.
	12	Post-FC1		56.86		
13	1	No-effect	2	AT		—Macroreentry around the right pulmonary veins. —Line from anterior mitral annulus to scar in the anterior LA wall including rotor.

					–Small reentry in the interatrial septum. –Macroreentry around the left pulmonary veins.
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–Small reentry

in anterior

mitral

annulus.

–Small reentry

in

anteroseptal

LA wall.

–ASL to

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Other 14

Pre-FC

56.06

15286

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AT

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					ablations.
					—Prophylactic
					CTI ablation.
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15		Post-FC3		58.63	on.
16	±	SR			—Perimitral flutter: ASL.

					– Small reentry in anterior LA wall. – Left PVI (gaps from previous procedure). – Typical atrial flutter.
17	1	AT			– Right PVI (rotor within RSPV antrum). – Small reentry in LSPV: focal ablation. – Macroreentry around the left pulmonary veins: left PVI. – Perimitral flutter: ASL. – AVNRT.
18	0		3	AT	– PVI (sites with SPT within the LIPV and RIPV antra, and in the ridge). – Reentry around posterior LA wall scar. – Perimitral flutter: ASL. – Focal AT from the vein of Marshall.

					–Focal AT from the anterior LA wall. –Prophylactic posterior LA box.
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Note: IQR, interquartile range (25th – 75th percentiles); Pre-FC, before first career; Post-FC, after first career

Supplementary Table 3: Professional, personal and understanding barriers for engaging in podiatry for both Pre-FC and Post-FC participants

	<u>Barrier</u>	<u>Group</u>	<u>Median (IQR)</u>	<u>Mean rank</u>	<u>U</u>	<u>z</u>	<u>p</u>	<u>Effect size (r)</u>
<u>Professional barriers</u>	<u>Geographical location</u>	<u>Pre-FC</u>	<u>3 (2-4)</u>	<u>50.13</u>	<u>1225.00</u>	<u>-1.148</u>	<u>0.251</u>	<u>0.11</u>
		<u>Post-FC</u>		<u>56.92</u>				
	<u>Lack of funding for second degree</u>	<u>Pre-FC</u>	<u>4 (2-5)</u>	<u>34.84</u>	<u>569.50</u>	<u>-0.706</u>	<u>0.480</u>	<u>0.07</u>
		<u>Post-FC</u>		<u>38.34</u>				
	<u>Financial support</u>	<u>Pre-FC</u>	<u>4 (2-5)</u>	<u>47.78</u>	<u>1108.50</u>	<u>-1.747</u>	<u>0.081*</u>	<u>0.16</u>
		<u>Post-FC</u>		<u>58.18</u>				
	<u>Cost of training</u>	<u>Pre-FC</u>	<u>4 (2-5)</u>	<u>48.41</u>	<u>1124.00</u>	<u>-1.267</u>	<u>0.205</u>	<u>0.12</u>
		<u>Post-FC</u>		<u>55.94</u>				
	<u>High workload</u>	<u>Pre-FC</u>	<u>4 (2-4)</u>	<u>54.4</u>	<u>1440.50</u>	<u>-0.904</u>	<u>0.366</u>	<u>0.08</u>
		<u>Post-FC</u>		<u>59.84</u>				
	<u>Perceived working conditions</u>	<u>Pre-FC</u>	<u>3 (2-4)</u>	<u>53.22</u>	<u>1367.00</u>	<u>-0.520</u>	<u>0.603</u>	<u>0.05</u>
		<u>Post-FC</u>		<u>56.3</u>				
<u>Personal Barriers</u>	<u>Lack prestige</u>	<u>Pre-FC</u>	<u>3 (2-4)</u>	<u>58.51</u>	<u>1149.50</u>	<u>-1.511</u>	<u>0.131</u>	<u>0.14</u>
		<u>Post-FC</u>		<u>49.66</u>				
	<u>Lack of representation (gender)</u>	<u>Pre-FC</u>	<u>2 (2-3)</u>	<u>52.86</u>	<u>1124</u>	<u>-0.762</u>	<u>0.446</u>	<u>0.07</u>
		<u>Post-FC</u>		<u>48.72</u>				
	<u>Lack of representation (ethnicity)</u>	<u>Pre-FC</u>	<u>2 (2-3)</u>	<u>52.27</u>	<u>1143.5</u>	<u>-0.554</u>	<u>0.579</u>	<u>0.05</u>
		<u>Post-FC</u>		<u>49.22</u>				
	<u>Perceived image</u>	<u>Pre-FC</u>	<u>2 (2-4)</u>	<u>53.76</u>	<u>1271</u>	<u>-0.384</u>	<u>0.701</u>	<u>0.04</u>
		<u>Post-FC</u>		<u>51.54</u>				
	<u>Perceived difficulty of profession</u>	<u>Pre-FC</u>	<u>3 (2-4)</u>	<u>53.76</u>	<u>1409</u>	<u>-0.812</u>	<u>0.417</u>	<u>0.08</u>
		<u>Post-FC</u>		<u>58.63</u>				

	<u>Perceived course difficulty</u>	<u>Pre-FC</u>	<u>3 (2-4)</u>	<u>57.09</u>	<u>1470.5</u>	<u>-0.332</u>	<u>0.740</u>	<u>0.04</u>
		<u>Post-FC</u>		<u>55.11</u>				
	<u>Lack of accessibility</u>	<u>Pre-FC</u>	<u>3 (2-4)</u>	<u>47.24</u>	<u>1085.5</u>	<u>-0.087</u>	<u>0.931</u>	<u>0.01</u>
		<u>Post-FC</u>		<u>47.72</u>				
	<u>Emotional burden</u>	<u>Pre-FC</u>	<u>3 (2-4)</u>	<u>54.13</u>	<u>1409</u>	<u>-0.254</u>	<u>0.800</u>	<u>0.02</u>
		<u>Post-FC</u>		<u>55.63</u>				
	<u>Outside obligations</u>	<u>Pre-FC</u>	<u>3 (2-5)</u>	<u>47.85</u>	<u>1120</u>	<u>-1.698</u>	<u>0.090</u>	<u>0.16</u>
		<u>Post-FC</u>		<u>57.83</u>				
<u>Understanding barriers</u>	<u>Shadowing after first career</u>	<u>Pre-FC</u>	<u>3 (2-5)</u>	<u>52.03</u>	<u>1078.5</u>	<u>-0.838</u>	<u>0.402</u>	<u>0.08</u>
		<u>Post-FC</u>		<u>47.35</u>				
	<u>Awareness of profession</u>	<u>Pre-FC</u>	<u>4 (2-4)</u>	<u>59.08</u>	<u>1364</u>	<u>-0.917</u>	<u>0.359</u>	<u>0.09</u>
		<u>Post-FC</u>		<u>53.65</u>				
	<u>Accessing information on profession</u>	<u>Pre-FC</u>	<u>3 (2-4)</u>	<u>59.86</u>	<u>1133.5</u>	<u>-1.766</u>	<u>0.077</u>	<u>0.16</u>
		<u>Post-FC</u>		<u>49.58</u>				
	<u>Limited information on profession</u>	<u>Pre-FC</u>	<u>3 (2-4)</u>	<u>57.66</u>	<u>1285</u>	<u>-0.955</u>	<u>0.340</u>	<u>0.09</u>
		<u>Post-FC</u>		<u>52.07</u>				
	<u>Career advisors lack of awareness</u>	<u>Pre-FC</u>	<u>4 (3-5)</u>	<u>51.23</u>	<u>599.5</u>	<u>-2.889</u>	<u>0.004</u>	<u>0.27</u>
		<u>Post-FC</u>		<u>36.13</u>				
	<u>Misconception of profession</u>	<u>Pre-FC</u>	<u>4 (2-5)</u>	<u>62.24</u>	<u>1164</u>	<u>-2.06</u>	<u>0.039*</u>	<u>0.19</u>
		<u>Post-FC</u>		<u>50.08</u>	<u>-</u>	<u>-</u>	<u>-</u>	

Note: IQR, interquartile range (25th – 75th percentiles); Pre-FC, before first career; Post-FC, after first career

Bold p values denotes significant difference between groups following Bonferroni adjustment (p = 0.008)

*Denotes p values that are approaching a significant difference between groups following a Bonferroni adjustment (p = .006)

ASL = anteroseptal line (line between mitral annulus and right superior pulmonary vein); AT = atrial tachycardia; AVNRT = atrioventricular nodal reentrant tachycardia; CTI = cavotricuspid isthmus; IVC = inferior vena cava; LA = left atrium; LIPV = left inferior pulmonary vein; LSPV = left superior pulmonary vein;

MAL = modified anterior line (line between mitral annulus and left superior pulmonary vein); PVI = pulmonary vein isolation; RIPV = right inferior pulmonary vein; RSPV = right superior pulmonary vein; STD = spatiotemporal dispersion; SVC = superior vena cava.

* Sites with spatiotemporal dispersion plus non-continuous fractionation were only mapped if no rotors were present or if rotor ablation was ineffective.

‡ Spontaneous conversion from AF to stable AT occurred after rotor mapping in the left atrium was completed (no rotors were found).